Adult advanced NSCLC · ALK or ROS1 indication confirmed Adults with advanced ALK-positive or ROS1-positive NSCLC. NCCP 00243a is previously treated ALK disease; 00243b is ROS1 disease; 00243c is first-line ALK disease. Confirm the exact indication and local access; a listed indication, licence and reimbursement are different facts. This guide does not cover paediatric lymphoma or inflammatory myofibroblastic tumour.

NCCP 00243 · VERSION 7

Crizotinib monotherapy

The usual adult starting dose is 250 mg twice daily, every day, with no planned rest week. Use the strength and frequency on your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

Oral ALK/ROS1 targeted therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Vision

Crizotinib

Report visual change; severe visual loss needs urgent assessment.

2 Heart and rhythm

Crizotinib

Slow pulse, rhythm changes or fluid-related swelling need assessment.

3 Breathing

Crizotinib

New cough or breathlessness can indicate serious lung inflammation.

4 Liver

Crizotinib

Liver tests and prompt reporting of jaundice or dark urine protect against serious injury.

5 Bowel and stomach

Crizotinib

Nausea, vomiting, bowel changes and severe abdominal pain need advice.

6 Kidneys and hydration

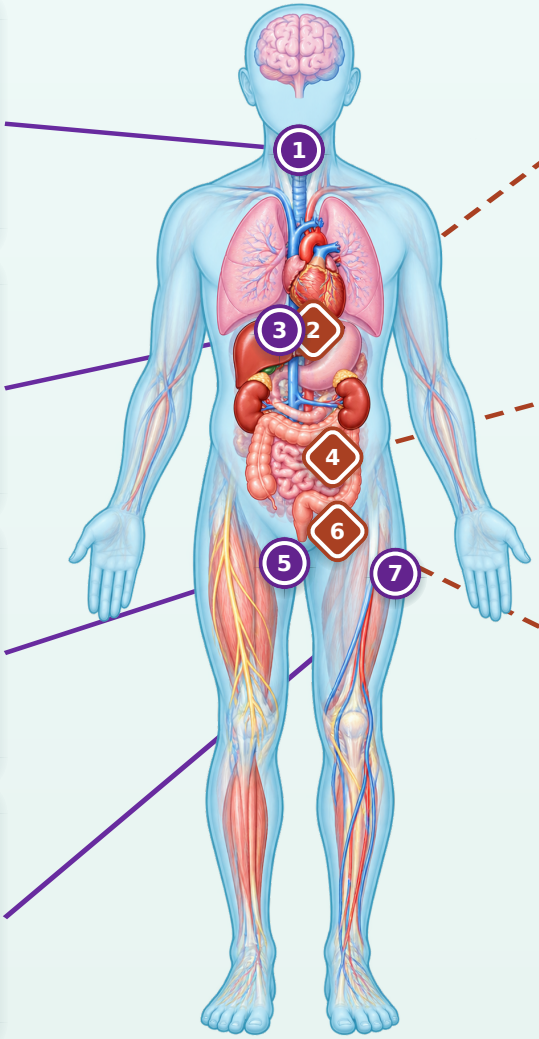
Crizotinib

Report poor drinking or reduced urine; kidney tests continue during treatment.

7 Skin and sunlight

Crizotinib

Rash and sun sensitivity can occur; use protection and report severe changes.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact your oncology team immediately for new or worsening breathlessness or cough, fainting, palpitations, severe dizziness, rapidly increasing swelling/weight, sudden or severe loss of vision, yellow skin/eyes, dark urine, fever/shivering or sudden illness, severe abdominal pain, or persistent vomiting/diarrhoea with poor drinking or little urine. Do not wait for the next appointment. Severe breathing difficulty, collapse or severe chest pain needs emergency help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

YOUR PRACTICAL TREATMENT PLAN

Know the next step.

Taking treatment

The usual adult starting dose is 250 mg twice daily, every day, with no planned rest week. Use the strength and frequency on your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

Swallow 200 mg/250 mg hard capsules whole with water, with or without food; do not crush, dissolve or open them. Other formulations have different instructions. If a dose is missed, take it when remembered only if the next dose is at least 6 hours away; otherwise skip it. Never double a dose. If you vomit after a dose, do not take an extra dose; take the next dose at its usual time.

Checks and follow-up

Before starting: confirm the tumour test, blood count, kidney/liver tests, chest imaging, symptoms and all medicines; ECG and electrolytes near the first dose. Liver tests are planned weekly for the first 2 months using current product information, then monthly; NCCP says every 2 weeks initially, so confirm the local plan. Blood count and kidney tests are monthly. ECG, electrolytes, pulse and blood pressure need a planned schedule and earlier review if unwell. NCCP lists monthly chest X-ray; other disease imaging follows your consultant plan.

The whole-treatment map shows the checks for this exact treatment setting.

Support between reviews

A tailored anti-sickness plan

Crizotinib has moderate-to-high oral sickness risk. Agree daily prevention and rescue treatment with your team. The chosen medicine must account for heart-rhythm and drug-interaction risks; do not add anti-sickness medicines or a prolonged steroid course yourself.

Food and medicine interactions

Avoid grapefruit/grapefruit juice and St John’s wort. Check prescription medicines, pharmacy medicines, vitamins and herbal products before taking them. Some antibiotics, antifungals, heart medicines and seizure treatments interact. Do not stop a prescribed medicine without a plan.

Doses and supply

Swallow 200 mg/250 mg hard capsules whole with water, with or without food; do not crush, dissolve or open them. Other formulations have different instructions. If a dose is missed, take it when remembered only if the next dose is at least 6 hours away; otherwise skip it. Never double a dose. If you vomit after a dose, do not take an extra dose; take the next dose at its usual time. Request repeat supply early and keep a current medicines list. A clinician-held dose needs clinician authorisation to restart.

Pregnancy, fertility and feeding

Discuss fertility preservation before treatment. Use an effective agreed contraception plan during treatment and for at least 90 days after the last dose; this applies to men and women. Oral contraceptives may be less effective. Tell the team promptly about pregnancy and avoid breastfeeding during treatment.

Vision, driving and sunlight

Report visual changes. Take care with driving or machinery if vision, dizziness or fatigue affects safety. Avoid prolonged sun exposure and use protective clothing and sunscreen.

Oncology daytime: Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital’s urgent-care plan. Emergency: 112 / 999.

Your complete treatment reference

https://sactcheck.com/patient/00243/

Public information only. No personal information is encoded.